

University Cancer Institute - Leukemia & BMT Clinic
OUTPATIENT PROGRESS NOTE

PATIENT: Kelly, Chris (MRN AML077)

DOB: October 23, 1984

DATE OF VISIT: March 15, 2024

SUBJECTIVE:

Mr. Kelly is a 39-year-old gentleman, now 21 months from the completion of his frontline chemotherapy for favorable-risk AML, who presents for a routine surveillance visit. He reports feeling "great, almost like it never happened." He has been back to work as a software engineer full-time for over a year. He notes his physical stamina is back to 100%, and he has resumed running and weightlifting. He reports no fevers, chills, new bruises, or other constitutional symptoms. He does mention occasional, mild "brain fog" or difficulty with short-term memory, especially at the end of a long workday, which he attributes to the after-effects of his treatment. He has no other active medical complaints.

RECAP OF CLINICAL COURSE:

Mr. Kelly was 37 years old and in peak physical health when he was diagnosed with AML in June 2022 after presenting with an unprovoked nosebleed and fatigue. His bone marrow biopsy confirmed AML with a **bZIP in-frame mutated CEBPA**, placing him squarely in the **ELN 2022 Favorable-risk** category. On June, 8 he underwent standard induction with **"7+3"** chemotherapy, achieved a complete remission, and subsequently completed three intensive cycles of consolidation with **High-Dose Cytarabine (HiDAC)**, with his final treatment in October 2022.

OBJECTIVE:

Vitals: T 36.8°C, HR 62, BP 118/75 mmHg.

General: Appears healthy, muscular, and well-nourished.

Exam: Unremarkable. No skin lesions, no lymphadenopathy, no organomegaly.

Labs (15-Mar-2024): CBC and CMP are entirely within normal limits. WBC 6.1, Hgb 15.5, Plt 310.

ASSESSMENT & PLAN:

Mr. Kelly remains in a durable, stringent complete remission from his favorable-risk AML. His prognosis is excellent, and at this stage, he can be considered cured of his leukemia, though we will continue surveillance for the full five years. Today's visit focused on his transition to long-term survivorship and addressing the unique needs of a young adult cancer survivor.

1. **AML Surveillance:** His risk of relapse is now very low (<10%). We will de-escalate the intensity of his monitoring.

- **Plan:** Transition follow-up visits to every 6 months. Continue with CBC checks every 6 months. We will plan for one final surveillance bone marrow biopsy at the 5-year mark post-treatment, or sooner if indicated.
2. **Long-Term Side Effects of Therapy:**
- **"Chemo Brain":** His report of mild cognitive slowing is a well-documented phenomenon. I reassured him that this is very common and often improves slowly over several years. We discussed cognitive rehabilitation strategies (e.g., list-making, mindfulness, regular exercise) and provided him with resources from our survivorship program.
 - **Fertility:** This is a critical issue that we have discussed previously. He underwent sperm banking prior to starting chemotherapy. Given that high-dose chemotherapy can cause permanent infertility, this was a crucial step.
 - **Plan:** Referred him to Reproductive Endocrinology for a formal fertility assessment (semen analysis) to determine if he has had any recovery of gonadal function. This will be important for his future family planning.
 - **Cardiovascular Health & Secondary Malignancy:** Discussed the small but real long-term risks associated with his chemotherapy. Emphasized the importance of a heart-healthy lifestyle and adherence to all future health screenings.
3. **Psychosocial Adjustment:** Mr. Kelly is well-adjusted, but we discussed the emotional "scanxiety" that can precede follow-up appointments. We reviewed our open-access policy to our social work and psychology teams should he ever feel the need for additional support.

He is a true success story and a model for recovery after intensive AML therapy. He was given a lab slip for his next appointment in 6 months and left the clinic in good spirits.